



OFFICIAL

NMHS MHS PROCEDURE

Falls Prevention and Management Procedure

Scope (Audience)	All NMHS Mental Health Services (MHS) Clinical Workers
Scope (Area)	All NMHS MHS Inpatient Areas
Summary	This procedure sets out the minimum requirements to ensure the delivery of safe patient care through prevention and management of falls to minimise patient harms across NMHS MHS, sites and services in accordance with the Australian Commission on Safety and Quality Health Care (ACSQHC) National Safety and Quality Health Service Standards – Comprehensive Care Standard .
Risk Rating	Medium
NSQHS Standards	Clinical Governance (Std 1) Comprehensive Care (Std 5) Communicating for Safety (Std 6)
Policy Sponsor	Co-Director Specialities MHSDHS
Policy Contact	Director of Nursing MHSDHS
Version	V1.0
Review Date	17/03/2029

COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

This procedure outlines the minimum requirements to ensure a delivery of safe patient care through prevention and management of falls to minimise patient harm across NMHS MHS sites and services. This procedure also includes consideration of risk mitigation, incident reporting and implantation of corrective actions under the [Work Health and Safety Act 2020](#) to provide a safe workplace.

2. Key Points

This procedure should be read in conjunction with the:

- [NMHS Falls Prevention and Management Policy](#)
- [Standard 5: Comprehensive Care of the National Safety and Quality Health Service \(NSQHS\) Standards](#)
- [WA Department of Health Post Fall Multidisciplinary Management Guidelines for WA Health Care Settings 2023 \(WA Guidelines\)](#)

Services with limited allied health staffing should consider interdisciplinary approaches to assessment of fall risk as per the [WA Guidelines](#).

3. Risk

Non-compliance with this policy will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Worker Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☒

4. Definitions

Terms	Definitions
Fall	An event which results in a person (patient, worker, member of public) coming to rest inadvertently on the ground, or other lower levels. This includes: • all syncopal events; • a patient rolls out of bed; • a patient is suspected of having put themselves on the floor; • where a fall is controlled by another person; and • where a baby falls from a raised service or a care giver's hold.
Patient	For the purpose of this procedure "patient" refers broadly to a patient, client, people with lived experience, and any other types of consumers who attend health care at NMHS Mental Health sites and services.
Worker	For the purpose of this procedure and as defined by the Work Health and Safety Act 2020 : A person who carried out work for NMHS in any

5. Procedure

5.1 Falls risk screening and assessment

- All patients admitted to an inpatient service will undergo a fall risk screening and assessment using a Falls Risk Assessment and Management Plan PMR98 Form (FRAMP) (**Appendix 1**).
- Falls Risk Assessment and Management Plan (FRAMP) is to be completed on admission or as soon as practicable after admission (within six (6) hours).
- Where services are available, all patients admitted to an inpatient service will be screened for potential review by:
 - **Medical Officer** refer to **Appendix 2: Medical Post Falls Guidelines** (inpatient falls)
 - **Occupational Therapist** refer to **Appendix 3: Occupational Therapy Post Fall Guidelines**
 - **Physiotherapist** refer to **Appendix 4: Physiotherapy Post Fall Guidelines**
 - **Pharmacist** refer to **Appendix 5: Pharmacy Post Fall Guidelines**
- Re-screening and assessment should be repeated when the patient is transferred from one ward/department to another; daily or when the patient's clinical condition changes; after a procedure requiring sedation or anaesthetic; or after a fall.
- The FRAMP shift-by-shift sign-off of interventions are to be completed by the appropriate worker (usually nursing) .
- At each screen, communication and information will be provided to the patient, nominated family member, support person, and/or carer and be documented in the FRAMP.
- Evaluation of falls prevention and management strategies and any changes are to be documented in the patient's medical record.

5.2 Falls prevention

5.2.1 Minimum interventions

Existing FRAMP minimum interventions will be reviewed shift by shift, on transfer to a different ward/department, post-fall, or when the patient's medical condition changes.

5.2.2 Individualised interventions

- Patients identified as being at risk of falls may require individualised interventions dependent on a collaborative and holistic assessment.

- Individualised interventions may be added by any member of the multidisciplinary team following discussion with the nurse in charge of care.
- Individualised interventions will be reviewed shift by shift, on transfer to a different ward/department, after a fall, or when the patient's medical condition changes.
- Individual patient functional assistance requirements prescribed assistive devices, and environmental modifications are to be documented within the FRAMP.
- Bed rails or any other forms of restraint are not recommended in either WA Health policy or national guidelines as a falls prevention strategy. There is potential harm to individuals when bed rails are used.
- Consideration can be given to the use of an ultra-low bed but not as a universal falls prevention solution and must not be provided inappropriately for mobile patients.

5.2.3 Communicating falls prevention information with patients and/or carers/legal guardian/nominated support person

- Communication with the patient's nominated family members, support persons, carers or legal guardian will be undertaken with full consent of the consumer unless clinical assessment indicates otherwise.
- All patients, their nominated family members, support persons, or carers will be informed about their fall risk and, where possible, involved in formulating their fall prevention and management plan.
- Where appropriate, patients, their nominated family members, support persons, or carers should be provided with falls education material information related to falls available on the SCGH public website [Falls Management](#).

5.3 Fall and post-fall management

5.3.1 Management of a person who is falling

If a patient fall cannot be avoided, the worker who witnesses the fall will attempt, if practicable, to protect the patient's head from impact and injury ensuring, as far as reasonably possible, that they do not put themselves at risk of injury in accordance with the [NMHS Manual Tasks \(Management of\) Policy](#) see [Appendix 1: Manual task risk and controls for "Patient Handling"](#).

5.3.2 Post fall management

- Following a fall, workers will commence the Nursing/Multidisciplinary 48 Hour Post-Fall Pathway (**Appendix 6**).
- Commence neurological or physical observations in accordance with the Nursing/Multidisciplinary 48-Hour Post-Fall Pathway (**Appendix 6**).
- Notify a Medical Officer of a patient fall with review to be completed within 4 hours, or within 30 minutes if fast-tracked, using the Post Fall Medical Assessment with reference to the [WA Guidelines](#).

- FRAMP re-screening, review of current prevention interventions, and implementation of additional interventions will be completed within six (6) hours.
- The allocated Nurse will document the fall in the patient's medical record.
- Referrals to the Occupational Therapist, Physiotherapist and Pharmacist will be completed within six (6) hours. For Older Adults this may include undertaking a functional assessment see **Appendix 7: MHS Functional Assessment Form (TMHMR910)**.
- Allied Health and other referred health professionals will complete and document post-fall assessments within two (2) working days (where services are available) with reference to the [WA Guidelines](#). Refer to **Appendix 8: MHS Physiotherapy Post Fall Review Form (PMR18K)**
- A Post Fall Multidisciplinary Huddle (PFMH) will be completed within 24-48 hours of the fall in a structure consistent with the [WA Guidelines](#) and documented in the patient's medical record.

5.4 Clinical communication

- Falls risk and incidents will be communicated in a clinical handover using the iSoBAR framework in accordance with the [WA Department of Health Clinical Handover Policy \(MP 0095/18\)](#).
- Falls risk will be documented on the Falls Risk Assessment and Management Plan PMR98 Form and recorded on PSOLIS/webPSOLIS in accordance with the [NMHS MHS Management of Clinical Alerts Policy](#).
- Consideration should be given to inclusion of relevant falls information in the TSDP and on iSOFT. A patient who has experienced a fall will be identified as a 'Person of Concern' for 24 hours (unless clinically indicated to be extended beyond this timeframe) in accordance with the [MHS Clinical Risk Assessment and Management Policy](#) and the [MHS Recognising and Responding to Acute Deterioration \(RRAD\) Physiological and Mental State Procedure](#).

5.5 Incident reporting

- The allocated Nurse, Nursing Shift Coordinator, Senior Registered Nurse, or Medical Officer should inform the patient's Next of Kin within 6 hours of a fall and in accordance with the [NMHS Open Disclosure Policy](#).
- The allocated nurse will notify a fall incident into the DATIX Clinical Incident Management System (DATIX-CIMS) by the end of their workday in accordance with the [NMHS Clinical Incident Management Policy 2019](#).
- The DATIX-CIMS senior staff investigator will, at minimum, invite the participating Occupational Therapist, Physiotherapist, and Pharmacist to provide third-party comments prior to determining contributing factors/recommendations.
- Injurious falls or complications of a fall that meet Severity Assessment Code (SAC) 1 criteria will be notified, investigated, and reported per SAC 1 requirements in accordance with the [NMHS Clinical Incident Management Policy 2019](#).

Any worker injury sustained during a fall will be reported as soon as practicable to the NMHS Health Safety and Wellbeing (HSW) and completing the [NMHS Hazard/incident Form](#) in accordance with the [NMHS Hazard/Incident Reporting and Investigation Policy](#).

5.6 Discharge planning

- Individual patient risk for falls, episodes of falls (if any) and their subsequent management and outcomes will be documented in the discharge summary in accordance with the [NMHS Discharge Summary Policy](#) and the NMHS MHS [Discharge Planning for Inpatient Procedure](#).
- Patient's identified as at risk for falls or for post-fall management following discharge will, where indicated, be referred to appropriate outpatient clinic/community service providers.
- Prior to discharge, patients, their nominated family members, support persons, and/or carers will receive falls prevention and management information tailored to their risk factors, well-being, and home environment.
- Medication review by a pharmacist and education of prescribed medications will be provided to patients prior to discharge.
- Patients who sustain a fall 48 hours before discharge will be provided with a completed [Health advice following a fall \(WA Health\)](#) information brochure

5.7 Home Visiting Health Professionals

The WA Health Multidisciplinary Post Fall Flowchart for Home Visiting Health Professionals (**Appendix 9 & 10**) should be used by all home-visiting workers to guide the assessment and management of patients who have fallen at home within the preceding 72 hours and have not received a medical assessment.

6. Governance

MHS Specialties Falls Prevention and Management Committee are responsible for:

- Development, endorsement and implementation of evidence-based falls prevention and management procedures within inpatient units.
- Collecting and reviewing falls data and identifying trends.
- Developing, implementing an evaluating improvement strategy as required.
- Escalating any risks or concerns to the Standard 5 Comprehensive Care Sponsor (or equivalent).
- Education and training for workers, patients as well as the community to increase awareness of falls risk and knowledge of falls prevention and management.
- Committee to ensure attendance by relevant services to ensure representation.

7. Monitoring and Evaluation

Compliance with this procedure is mandatory. Evaluation will be undertaken annually in accordance with the MHSDHS Safety, Quality and Performance Physical Health Care Audit with outcomes and recommendations reported to services and leadership committees.

This procedure will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed every three (3) years using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

8. Related Documents

Legislation	• Work Health and Safety Act 2020
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WA Health mandatory policies	• Clinical Handover Policy (MP 0095/18) • Post Fall Multidisciplinary Management Guidelines for WA Health Care Settings 2023 • Language Services Policy (MP 0051/17)
NMHS Policy documents	• NMHS Hazard/Incident Reporting and Investigation Policy • NMHS Manual Tasks (Management of) Policy • NMHS Work Health and Safety (WHS) Policy • NMHS Clinical Documentation Policy • NMHS Clinical Incident Management Policy • NMHS Open Disclosure Policy
Standards	• NSQHS Comprehensive Care Standard

9. Resources

- [‘Patient resources – Falls Management’](#)
- [Post Fall Multidisciplinary Management Guidelines for Western Australian Health Care Settings 2023](#)

10. Document Control

Document Version Control			
Date	Version	Author	Notes
17/03/2026	V1.0	Falls Prevention and Management Committee	Initial endorsement

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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Appendix 1: Falls Risk Assessment and Management Plan (FRAMP)

Please use I.D. label or block print

NORTH METROPOLITAN HEALTH SERVICE MENTAL HEALTH FALLS RISK ASSESSMENT AND MANAGEMENT PLAN (FRAMP)	SURNAME	UMRN	
	GIVEN NAMES	DOB	GENDER
	ADDRESS		POSTCODE
	TELEPHONE		
WARD			
DOCTOR			

On this shift has the patient:
 Been admitted or transferred from another ward; or
 Had a fall; or
 Medically deteriorated or improved?

☐ YES to ANY
☐ NO to ALL

Initial Screen ☐ Admitted ☐ Ward Transfer ☐ Post Fall ☐ Medical Condition Change

Does the patient meet any of the following: *Circle Yes or No*

1. Had a fall in the past 12 months?	YES / NO
2. Unsteady when walking/transferring or uses a walking aid?	YES / NO
3. Confused, known cognitive impairment or incorrectly answers any of the following: Age, Date of birth, Current year and Place?	YES / NO
4. Experienced effects of psychoactive/other medication (drowsiness, hypotension, postural hypotension, blurred vision)	YES / NO
5. Has urinary or faecal frequency/urgency or nocturia?	YES / NO

Name: _____ Designation: _____ Ward: _____

Date: _____ Time: _____ Signature: _____

Re-Screen 1 ☐ Admitted ☐ Ward Transfer ☐ Post Fall ☐ Medical Condition Change

Does the patient meet any of the following: *Circle Yes or No*

1. Had a fall in the past 12 months?	YES / NO
2. Unsteady when walking/transferring or uses a walking aid?	YES / NO
3. Confused, known cognitive impairment or incorrectly answers any of the following: Age, Date of birth, Current year and Place?	YES / NO
4. Experienced effects of psychoactive/other medication (drowsiness, hypotension, postural hypotension, blurred vision)	YES / NO
5. Has urinary or faecal frequency/urgency or nocturia?	YES / NO

Name: _____ Designation: _____ Ward: _____

Date: _____ Time: _____ Signature: _____

Re-Screen 2 ☐ Admitted ☐ Ward Transfer ☐ Post Fall ☐ Medical Condition Change

Does the patient meet any of the following: *Circle Yes or No*

1. Had a fall in the past 12 months?	YES / NO
2. Unsteady when walking/transferring or uses a walking aid?	YES / NO
3. Confused, known cognitive impairment or incorrectly answers any of the following: Age, Date of birth, Current year and Place?	YES / NO
4. Experienced effects of psychoactive/other medication (drowsiness, hypotension, postural hypotension, blurred vision)	YES / NO
5. Has urinary or faecal frequency/urgency or nocturia?	YES / NO

Name: _____ Designation: _____ Ward: _____

Date: _____ Time: _____ Signature: _____

Confirm previously assessed interventions are in place as per Shift by Shift check on page 3.

YES to ANY
 Turn to page 2
 Complete Risk Assessment and Individualised Interventions.

NO to ALL
 Turn to page 3 and check Minimum Interventions are in place.

PMR 98 FALLS RISK ASSESSMENT AND MANAGEMENT PLAN (FRAMP)

Page 1 of 4

Appendix 1: Falls Risk Assessment and Management Plan (FRAMP) (Pg2)

DATE				
RISK ASSESSMENT and INDIVIDUALISED INTERVENTIONS		Initial Screen	Re-Screen 1	Re-Screen 2
MOBILITY RISKS Does the patient:		If risk identified initial box		
Require assistance with mobility/transfer?				
Have poor coordination, balance, gait or uncorrected visual impairment?				
FUNCTIONAL ABILITY RISKS				
Is the patient unsteady, disorganised or require assistance when attending to ADLs?				
INTERVENTIONS		<i>Initial if appropriate for patient</i>		
Assess, document and provide mobility aids and level of assistance required.				
Discuss and confirm with the patient what level of assistance they require (including mobility aids), and/or their need to call and wait for assistance.				
Refer to Physiotherapist for a comprehensive mobility assessment.				
Refer to Occupational Therapist (OT) for functional assessment.				
MEDICATIONS/MEDICAL CONDITION RISKS		If risk identified initial box		
Some medications are associated with falls. Has the patient been prescribed:				
-Psychoactive medication e.g. benzodiazepines, antipsychotics, antidepressants?				
-New or old medication that may affect their blood pressure?				
Does the patient take more than 5 medications of any sort?				
Does the patient report dizziness or presented following a fall/collapse?				
Does the patient misuse illicit drugs or alcohol?				
INTERVENTIONS		<i>Initial if appropriate for patient</i>		
Liaise with Medical Officer (MO) or Pharmacist for review of medication associated with falls.				
If reporting dizziness, check lying/standing blood pressure. If a postural drop >20mmHg systolic or >10mmHg diastolic present, discuss plan of care with MO.				
Educate patient to stand up slowly and wait until dizziness resolves before mobilising. If dizziness persists, discuss plan of care with MO.				
Commence detoxification program.				
COGNITIVE STATE RISKS Does the patient have:		If risk identified initial box		
Previous delirium or known diagnosis of dementia?				
New or worsening memory impairment, confusion or disorientation?				
Drowsiness, is easily distracted, withdrawn or depressed?				
INTERVENTIONS		<i>Initial if appropriate for patient</i>		
Establish a baseline cognitive screen eg Abbreviated Mental Test (AMT).				
If result abnormal (e.g. MMSE <24) refer to treating team.				
Increase assistance and supervision with ADL's				
If agitated commence behaviour observation chart to assist behaviour management plan.				
Consider a low-low bed				
Set an alarm system in place to alert when patient is trying to get up unaided.				
Re-orientate patient.				
Increase frequency of patient checks to pro-actively attend to patient needs.				
CONTINENCE/ELIMINATION RISKS Does the patient:		If risk identified initial box		
Require assistance with toileting?				
Have constipation, urinary or faecal frequency/urgency or nocturia?				
INTERVENTIONS		<i>Initial if appropriate for patient</i>		
Monitor/record toileting needs to check frequency, retention or constipation. Use site specific documentation.				
Review toileting needs with patient daily including frequency, patients requirement for continence/ toileting aids and assistance required to access toilet facilities.				
Complete urinalysis. If abnormal, discuss with MO if MSU indicated.				
PATIENT REQUIRES INTERVENTIONS OTHER THAN ABOVE (SEE PAGE 4)				

Appendix 1: Falls Risk Assessment and Management Plan (FRAMP)

Please use I.D. label or block print

NORTH METROPOLITAN HEALTH SERVICE MENTAL HEALTH FALLS RISK ASSESSMENT AND MANAGEMENT PLAN (FRAMP) WARD _____ DOCTOR _____	SURNAME _____		UMRN _____	
	GIVEN NAMES _____		DOB _____	GENDER _____
	ADDRESS _____		POSTCODE _____	
			TELEPHONE _____	

MINIMUM INTERVENTIONS To be implemented for ALL patients as appropriate																		
- Provide ongoing orientation for patient to bed area, toilet facilities and ward. - Ensure frequently used items including mobility aids are within easy reach of patient. - Encourage patient to use their aids such as glasses or hearing aids. - Adjust bed and chair to appropriate height for patient. - Minimise prolonged bed-rest as it contributes to negative cardiovascular and muscle effects that may lead to falls. - Remove clutter and obstacles from room. - Provide adequate lighting according to patient activities/needs. - Encourage patient to take adequate fluids and nutrition. - Optimise footwear where possible- discourage walking in socks/compression stockings or ill-fitting footwear. Bare feet (if there is no infection risk) and non-slip socks are acceptable. - Educate that all inpatients are at increased risk of falling due to injury / illness / medications.																		

SHIFT BY SHIFT CHECK If the patient has had a FALL or MEDICAL CONDITION CHANGE or WARD TRANSFER re-screen on page 1																		
Instructions: Please date and initial below to confirm which interventions are implemented each shift.																		
Week 1	Date / /			Date / /			Date / /			Date / /			Date / /			Date / /		
	AM	PM	ND	AM	PM	ND	AM	PM	ND	AM	PM	ND	AM	PM	ND	AM	PM	ND
Minimum Interventions ONLY OR																		
Minimum AND Individualised Interventions																		
Week 2	Date / /			Date / /			Date / /			Date / /			Date / /			Date / /		
	AM	PM	ND	AM	PM	ND	AM	PM	ND	AM	PM	ND	AM	PM	ND	AM	PM	ND
Minimum Interventions ONLY OR																		
Minimum AND Individualised Interventions																		
Week 3	Date / /			Date / /			Date / /			Date / /			Date / /			Date / /		
	AM	PM	ND	AM	PM	ND	AM	PM	ND	AM	PM	ND	AM	PM	ND	AM	PM	ND
Minimum Interventions ONLY OR																		
Minimum AND Individualised Interventions																		
Week 4	Date / /			Date / /			Date / /			Date / /			Date / /			Date / /		
	AM	PM	ND	AM	PM	ND	AM	PM	ND	AM	PM	ND	AM	PM	ND	AM	PM	ND
Minimum Interventions ONLY OR																		
Minimum AND Individualised Interventions																		

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Appendix 1: Falls Risk Assessment and Management Plan (FRAMP) (Pg4)

Please use I.D. label or block print

NORTH METROPOLITAN HEALTH SERVICE MENTAL HEALTH FALLS RISK ASSESSMENT AND MANAGEMENT PLAN (FRAMP) WARD _____ DOCTOR _____	SURNAME _____		UMRN _____	
	GIVEN NAMES _____		DOB _____	GENDER _____
	ADDRESS _____			POSTCODE _____
				TELEPHONE _____

OTHER INDIVIDUALISED INTERVENTIONS				
Document other individualised interventions below.				
Interventions can be added by any member of the multidisciplinary team when discussed with the nurse in charge of care – e.g. Nurses, Allied Health, Medical Officer, Pharmacists				

Name and Designation	Date	Intervention	Date actioned and by whom	Date ceased and by whom

COMMUNICATION AND INFORMATION TO PATIENTS AND CARERS
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This section is for **patients identified at risk of falls**.

At each screen provide updated information about the risks for falling and plan care in partnership with patient and carer. If unable to discuss e.g. confused/low GCS and no carer, then tick unable.

	Date Discussed	Staff Member Name	Staff Member Signature	Whom Falls Risk Was Discussed With
Initial Screen	/ /			☐ Patient ☐ Carer ☐ Unable
Re-Screen 1	/ /			☐ Patient ☐ Carer ☐ Unable
Re-Screen 2	/ /			☐ Patient ☐ Carer ☐ Unable

Important Practice Points These patients need particular care managing their falls risk. - Patients on anticoagulant, antiplatelet therapy and/ or patients with a known coagulopathy are at an increased risk of intracranial haemorrhage from falls. -Alcohol dependent persons, people with liver disease and people with bleeding disorders are considered coagulopathic. - NB. Refer to local post-fall management procedure for more information. - Patients who are known to be osteoporotic or who have suffered low trauma fractures in the past are at increased risk of sustaining a fracture even from mild falls. - Consider discussing with the team, vitamin D supplementation (Cholecalciferol 1000units/day) for those patients with longer lengths of stay, vitamin D level < 80nmol/L or whom reside in residential care.

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DO NOT WRITE IN MARGIN

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Appendix 2: Medical Post Fall Guidelines

Medical practice guidelines

(Inpatient falls)

Introduction – the facts

- Once a patient has had one fall in hospital, they are at risk of having more falls.
- All falls are to be treated seriously by staff as often a fall is an indication of an underlying problem that can be treated.
- Even relatively minor falls in older people can lead to death or significant injury.

Protocol

- Every patient experiencing a fall in hospital requires a timely medical review (urgent if deteriorating, within 30 minutes if injured, and within 4 hours for most other falls).
- Services without resident medical staff should follow local escalation processes including use of the Emergency Telehealth Service (ETS).
- Responding to the fall incident requires the provision of immediate first aid, longer-term care, and active addressing of falls risk factors to prevent future falls.
- Consider using a proforma to assist with meeting examination requirements (See [Appendix 2](#)).

History

- Talk to the patient about the fall and symptoms arising from the fall.
- Review medical entries in the patient's health care record and medication chart to identify factors that may put the person at risk of falling, or of having an injury from the fall.
- Establish the patient's baseline mobility and cognitive state and determine whether it has changed post fall.
- Specifically, document whether the person is on Warfarin, Enoxaparin (Clexane), Heparin, Apixaban (Eliquis), Rivaroxaban (Xarelto), Dabigatran (Pradaxa), Prasugrel (Effient) and Ticagrelor (Brilinta), Fondaparinux, Lepirudin, Bivalirudin, Aspirin, Clopidogrel, Aspirin and Dipyridamole (Asasantin®) or other anticoagulant/antiplatelet medication.
- Patients with chronic liver disease or haematological disorders may also be coagulopathic.

Examination

- Examination should always take place, even if you must wake the patient.
- The examination should identify any injury sustained. When examining a patient, be aware that they may not draw attention to all their injuries (particularly if cognitively impaired).
- Do not allow the patient to be moved until head, cervical spine and hip injuries have been ruled out. Spinal precautions must be used if the patient has GCS <13 or a neck injury is suspected.
- The examination should also seek to identify the immediate underlying causes of the fall (infection, arrhythmia, stroke, hypotension, other acute illness).

Appendix 2: Medical Post Fall Guidelines (Pg2)

- The examination should include:
 - Check pulse and blood pressure (when appropriate check postural drop).
 - Assess level of consciousness and document Glasgow Coma Scale.
 - Talk to the patient – assess for confusion (delirium or dementia). Document AMT4 (age, DOB, current year, place).
 - Examine the head, neck, spine, hips, and limbs to identify sites of tenderness/swelling/deformity (for example a shortened, externally rotated leg may indicate a hip fracture).
 - Neurological examination including speech, pupil size, eye movements, facial asymmetry, power, sensation, and plantar responses.
 - If there are no obvious features of hip fracture, ensure hip range of movement is pain free, and as soon as is practicable ensure weight bearing is also pain free.
 - Assess post-fall mobility.

Investigations

- Order relevant investigations and ensure the results are checked and documented in the patient's health care record.
- Exclude intracranial haemorrhage and fractures.

1. Is a CT head scan required?

a). If the patient has hit their head?⁸

This decision should be individualised and based on their risk of injury. There is no specific research determining the optimal pathway for inpatients. The National Institute for Health Care and Excellence (NICE) guidelines developed for Emergency Departments provide useful criteria for clinicians to assist decision-making. These are reproduced below in Figures 1 and 2.

Sites without available CT scanning should utilize local pathways and consultation services. Deterioration in neurological observations undertaken by nursing staff is a trigger for CT scanning.

Fig 1. When to perform a CT head scan within 1 hour⁸

For adults who have sustained a head injury and have any of the following risk factors, perform a CT head scan within one hour of the risk factor being identified:

- GCS less than 13 on initial assessment.
- GCS less than 15 at 2 hours after the injury on assessment.
- Suspected open or depressed skull fracture.
- Any sign of basal skull fracture (haemotympanum, 'panda eyes', cerebrospinal fluid leakage from the ear or nose, Battle's sign).
- Post-traumatic seizure.
- Focal neurological deficit.
- More than one episode of vomiting.

Appendix 2: Medical Post Fall Guideline (Pg3)

Fig 2. When to perform a CT head scan within 8 hours⁸

For adults with **any** of the following risk factors who have experienced some loss of consciousness or amnesia since the injury, perform a CT head scan within 8 hours of the head injury:

- Age 65 years or older.
- Any history of bleeding or clotting disorders.
- Current anticoagulation treatment.
- Dangerous mechanism of injury (a pedestrian or cyclist struck by a motor vehicle, an occupant ejected from a motor vehicle or a fall from a height of greater than one metre or five stairs).
- More than 30 minutes' retrograde amnesia of events immediately before the head injury.

b). If patient had a witnessed fall and did NOT hit their head and they do NOT need a CT head scan?

- CT Head required if signs of neurological impairment develop.

c). Unwitnessed fall with no signs of head injury?

Consider CT head scan within 8 hours if:

- Cognitively impaired.
- Neurological deterioration on nursing observations.
- On current anticoagulant treatment.

Whether a CT head scan will alter patient management and patient/carer preferences should be considered; for example, would the patient be considered appropriate for neurosurgical intervention? This dialogue should be documented in the patient's health care record and discussed with the treating specialist.

2. Is a CT cervical spine scan required?⁸

a) For patients with a head injury:

NICE guidelines⁸ make the following recommendations in relation to cervical CT requests:

Fig 3. Risk factors indicating CT cervical spine within 1 hour⁸

A cervical spine CT should be arranged within one hour for all adults who have sustained a head injury and have any of the following risk factors:

- GCS less than 13 on initial assessment.
- The patient has been intubated.
- Plain X-rays are technically inadequate (for example the desired view is unavailable).
- Plain X-rays are suspicious or abnormal.
- A definitive diagnosis of cervical spine injury is needed urgently (for example before surgery).
- The patient is having other body areas scanned for head injury or multi-region trauma.
- The patient is alert and stable, there is clinical suspicion of cervical spine injury and any of the following apply:
 - Age 65 years or older.
 - Dangerous mechanism of injury (fall from a height of greater than 1 metre or 5 stairs).
 - Focal peripheral neurological deficit.
 - Paraesthesia in the upper or lower limbs.

Appendix 2: Medical Post Fall Guideline (Pg4)

Fig 4. Assessing range of neck movement safely⁸

For adults who have sustained a head injury and have neck pain or tenderness but no indications for a CT cervical spine scan, perform three-view cervical spine X-rays within one hour if either of these risk factors are identified:

- *It is not considered safe to assess the range of movement in the neck.*
- *Safe assessment of range of neck movement shows that the patient cannot actively rotate their neck to 45 degrees to the left and right.*

Be aware that in adults who have sustained a head injury and in whom there is clinical suspicion of cervical spine injury, range of movement in the neck can be assessed safely before imaging only if there are no high-risk factors and at least one of the following low-risk features apply. The patient:

- *Was involved in a simple rear-end motor vehicle collision.*
- *Is comfortable in a sitting position.*
- *Has been ambulatory at any time since injury.*

Treatment

- Implement treatment as appropriate (for example resuscitation, immobilisation, pain relief).
- If patient has sustained significant injuries, inform the patient's consultant (or on call consultant after hours). If intracranial haemorrhage is confirmed, also urgently consult the neurosurgical registrar or consultant on call.
- If patient is unstable, return as often as deemed clinically appropriate to review.
- Implement appropriate actions to prevent a recurrence of a fall and communicate these to relevant staff.
- Review for high-risk medications. If clinical evidence for head injury, withhold anticoagulants until CT head scan is available.
- Inform the relevant medical team for follow-up.
- In the case of significant injuries, the doctor should inform the Next of Kin (NOK) if patient consents. Nursing staff will inform NOK about less serious falls.
- Consider referral to relevant members of the multidisciplinary team for further assessment and interventions as required

Stop and consider

- Have head, cervical spine and hip injuries been adequately ruled out? Do not allow patient to be moved until you have done so.
- Spinal precautions need to be used if the patient has GCS <13 or a neck injury is suspected.
- Has a new medical problem, for example sepsis, been adequately ruled out or treated?
- Does the patient have delirium or dementia, and is management in accordance with best practice?

Document and handover

- Documentation in the patient's health care record is vital. Sites may use a separate medical post-fall document, which should be used according to local policy.
- Communicate with relevant staff.

Appendix 2: Medical Post Fall Guideline (Pg5)

- If you have any doubts about appropriate investigations and management, contact the appropriate senior medical person and after hours, the afterhours registrar or medical officer on call.
- Consider using medical proforma for post fall assessment (see [Appendix 2](#)).

Medical treating team within 48 hours of the fall:

- Follow up any urgent investigations.
- Review medications e.g., on sleeping tablets – consider weaning off.
- Collaborate with the multidisciplinary team to identify all the patient's falls risk factors.
- Formulate an individualised management plan to address these.
- Assess for delirium and manage.
- Ensure the fall is mentioned on the discharge summary.

Discharge

The patient and carer should be provided with written and verbal information about falls. Discuss with the patient and carer about the risks of clinical deterioration, further falls and ensure that they have understood.

If discharged within 48 hours of a fall a "[Health Information After a Fall](#)" leaflet and discussion should be provided to ensure that the patient and carer have an understanding of the signs and symptoms of any clinical deterioration and where to seek help.

***Note: If the following occur, the fall must be reported to the coroner:**

- a fall that resulted in the patient's death.
- a fall that was sustained within 48 hours prior to the patient's death.

Occupational therapy guidelines

Introduction

Post inpatient fall the Occupational Therapist's role is to complete an analysis of the inpatient fall, identifying the patient's falls risk factors and how they contributed to the fall. Assessment, intervention, and recommendations should relate to the core Occupational Therapy areas of practice.

The Occupational Therapist, when completing an inpatient post fall assessment, focuses on:

- Patient's independence in Activities of Daily Living (ADLs);
- Patient's Vision;
- Patient's Cognition and;
- Review of the Environment (Hospital).

Additional supporting information can be found in [Appendix 3](#). This is not an exhaustive list of assessments and interventions. They are a guide to what could be considered in the post fall analysis. Not all interventions are suited to all patients. Occupational Therapy specific interventions should be patient-centred and match individual needs.

Standard

The Occupational Therapist should assess all patients who have had a fall whilst an inpatient within [two working days](#) where possible or as per local policy.

These reviews can take place in person or via remote review on telehealth.

Referral process post inpatient fall to occupational therapy

Nursing/medical or allied health can refer to Occupational Therapy directly or via e-referrals for an inpatient post fall assessment. The Occupational Therapist can also obtain referrals via journey board meetings, screening processes or by liaising directly with nursing or medical staff.

Post fall occupational therapy assessment and interventions

The Occupational Therapy inpatient falls assessment needs to consider the following:

1. Falls Analysis:

- Background information surrounding the fall, the mechanism of injury, where did the fall occur, when did the fall occur and what activity was being undertaken at the time of the fall.

2. Falls Risk Factor Identification, Stratification and Occupational Therapy Interventions:

- The risk factors and interventions focus on the core occupational therapy areas (see above) and how they relate to the inpatient's day-to-day function on the ward. Each identified risk factor should be addressed.
- All patients and their carers should be provided with written and verbal tailored falls prevention education.

Appendix 3: Occupational Therapy Post Fall Guidelines (Pg2)

- Intervention should also consider post-discharge needs and outpatient referrals especially for high-risk population i.e., adults experiencing frequent falls, adults with Parkinson's Disease, cognitive impairment etc. Please refer to [Appendix 3](#).

Documentation

Complete the falls analysis and identify the falls risk factors/interventions in the areas of ADL including the effect of vision, cognition, and the environment. Document your findings clearly and concisely in the patient's health care record or as per local documentation requirements.

- Minimum documentation should include:
 - Date & time of fall (if known).
 - Date & time seen by Occupational Therapist.
 - Analysis of fall: including risk factors identified.
 - Interventions and recommendations for falls risk minimisation: including consideration of the impact of the environment, ADL, vision, and cognition on falls risk if appropriate.
- The Occupational Therapy sticker for the patient's health record ([Appendix 3](#)) provides an example of iSoBAR documentation that could be utilised.
- If any further intervention is required from an Occupational Therapist, ensure this is documented in the health care record as required.

Physiotherapy post fall guidelines

Introduction

The role of Physiotherapy is integral to the multidisciplinary management and care of patients who are at risk of falling or have fallen.

Supporting information and resources are available in [Appendix 4](#)

Referral process post inpatient fall to physiotherapy

Nursing/Medical or Allied Health can refer to Physiotherapy for a patient falls assessment following an inpatient fall. The Physiotherapist can also obtain other referrals via journey board meetings, screening processes or by asking nursing or medical staff.

Following the notification of an inpatient fall, a Physiotherapist should review the patient within [two working days](#) where possible or as per local policy.

These reviews can take place in person, or via remote review on telehealth.

The following guidelines are recommended:

Assessment

- Liaise with relevant nursing staff and refer to the patient's health care records to investigate details, nature of, and events preceding the fall.
- Determine if the patient has had a review by a Medical Officer, Senior Nurse, or Nurse Practitioner (NP).
- Review the results of any medical investigations performed (for example X-ray and CT scans).
- Confirm if the patient has medical clearance to mobilise or is restricted to 'Rest in Bed'. Ideally, the Medical Officer has documented this prior to assessing and mobilising the patient. However, to prevent delays, a verbal approval may be sought. Document the name and designation of the Medical Officer providing the orders in the patient health care record.

As a minimum the following should be assessed:

Subjective Assessment

- If not known from a prior Allied Health Assessment, include previous level of functioning and mobility (including level of assistance required, walking aid use, cognition).
- Involve carers (with patient consent where possible) in assessments, especially for patients with cognitive impairment.
- Identify any previous falls history and/or current fall risk factors.
- Home environment.
- Discuss the fall with the patient, if they can recall the fall, to determine their insight into what happened and to gather information regarding their cognition.

Appendix 4: Physiotherapy Post Fall Guidelines (Pg2)

Comprehensive Objective Assessment

- When medically stable, the patient should receive a comprehensive assessment. This may include a mobility, strength, vestibular, balance assessment and any other assessment as determined appropriate at that point in care.
- If the patient is unable to participate in the assessment due to pain, difficulty in weight-bearing, acute confusion, or another reason, the Physiotherapist should refer to the Medical Officer for further review/investigations. A comprehensive Physiotherapy review to occur post results of relevant medical investigations.

Comprehensive Physiotherapy assessment should include the following:

- Consider assessment of postural Blood Pressure if this is a possible contributing factor to the fall.
- Consider basic cognition screen such as Abbreviated Mental Test (AMT)⁴.
- Current mobility and level of assistance required, including walking aid requirements.
- Balance assessment as appropriate
 - As a minimum standard, sitting and standing balance.
 - If the patient is unable to stand, an assessment of sitting balance should be completed.
 - Standing balance should be assessed in progressively more challenging positions e.g., timed unsupported stance with eyes open and closed, with feet apart, feet together, step stance, tandem stance, and single leg stance.
 - Dynamic standing balance should also be assessed, if appropriate.
- Assessment and outcome measures determined appropriate at that point in care (e.g., manual muscle testing, Timed Up and Go, Berg Balance Scale, De Morton Mobility Index, repeated sit to stand and step tests, vestibular assessment).
- The 2022 World Falls Guideline⁷ recommend using Gait Speed (GS) for predicting falls risk. For risk stratification they recommend use of GS, with a cut-off value of < 0.8 m/s based on its predictive ability and simplicity. This is measured at usual pace over 4 metres, with a cut off value, of < 0.8 m/s indicative of a falls risk.

Intervention

- According to assessment findings, and in collaboration with the multidisciplinary team, eliminate/control risk factors to reduce the risk of falls and harm from falls.
- In partnership with the patient and their carer/family, provide information about falls risk factors, risk reduction strategies, physiotherapy treatment, and the ongoing management plan. (Ensure patient consents to family/care involvement if possible).
- Supply equipment if required (e.g., walking aids).
- If the patients' shoes/slippers do not meet the criteria of a ['safe shoe'](#), request safe footwear or the patient is to wear non slip socks if possible. Check for local footwear guidelines/policy.
- Continue/commence patient rehabilitation (if indicated) incorporating specific balance and muscle strengthening components.
- Monitor for any behavioural or thinking changes during subsequent interactions with the patient that could indicate a delirium. Escalate any concerns to the Medical Officer.
- Where indicated, support staff with the new mobility/manual handling recommendations.

Appendix 4: Physiotherapy Post Fall Guidelines (Pg3)

- Communicate your interventions and recommendations, including level of assistance required to relevant staff (to enable the updating of nursing care plans and to highlight the patient as a Falls Risk).
- Ensure the “At risk of falling” alert sign above the patient’s bedside as per local guideline/policy.
- Consider providing appropriate “Stay on Your Feet WA” written resources, or available local resources on relevant falls risk prevention.

Documentation

- Document the patient has medical clearance to mobilise or is restricted to ‘Rest in Bed’. Ideally, the Medical Officer has documented this prior to assessing and mobilising the patient. However, to prevent delays, a verbal approval may be sought. Document the name and designation of the Medical Officer providing the orders in the patient health care record.
- Clearly and concisely document:
 - Clinical assessment findings and interventions completed.
 - Falls risk status.
 - Strategies and recommendations to reduce risk and harm from further falls.
 - Falls prevention education that was provided.
 - Handover of the patient’s mobility falls risk status and falls prevention recommendations that were provided to nursing staff and relevant multidisciplinary team.
- Update the locally endorsed Falls Risk Assessment Tool including the patient’s falls risk status.
- Where applicable, update patient’s mobility chart, local post fall checklist, and any other site-specific communication tools, as per local guideline/policy.

Discharge planning

- Ensure the history and management of the inpatient fall and falls risk is handed over and included on transfer and discharge documentation.
- Identify post-discharge rehabilitation and falls-prevention needs, completing timely referrals and handovers to appropriate local community services (including Physiotherapy, Occupational Therapy, Nursing, and Falls Clinics or as outlined as per local services discharge).

Further information is available at [Appendix 4](#).

Appendix 5: Pharmacist Post Fall Guidelines

Pharmacy post fall guidelines

Introduction

Medications are recognised as a major contributor to falls, and pharmacists can play an active role in reviewing medication regimens to optimise therapy and minimise the likelihood of further falls.⁹ Pharmacist-led medication reviews have an important role to play in improving prescribing safety. Medication reviews focused on falls prevention involve striking a balance between minimising medicines associated with falls and effectively treating medical conditions. This includes taking a comprehensive approach to reviewing the patient's clinical condition, screening for medications most likely to be implicated, and collaborating with Medical Officers (MO), patients and their caregivers to implement interventions to reduce the risk of falls and harm from falls.^{9,10}

Pharmacists should review patients within [two working days](#) of a fall, which may occur in person or via remote review or telehealth. Pharmacists should also, if possible, attend the post-fall huddles (if facilities undertake these).

Please see [Appendix 5](#) for resources that may assist pharmacists in their medication reviews.

Initial assessment and review

The initial assessment will consist of a clinical review covering at a minimum:⁹⁻¹¹

- Review of patient's current clinical condition, medication and local falls risk assessment and plan.
- Review of documentation related to the patient's clinical condition at the time of the fall.
- Review of potential issues regarding falls-related medication management.
- Assessment for pharmacokinetic and pharmacodynamic medication interactions that could be contributing to increased falls risk:
 - Medications directly contributing to falls such as side effects of a medication, for example orthostatic hypotension, sedation, impaired balance/coordination/reaction time, cognitive changes, hypoglycaemia, or hypotension secondary to levodopa administration.
 - Medications indirectly contributing to falls such as symptoms of under- or untreated conditions, for example tremor secondary to sub-optimal medication regimen in Parkinson's disease.
- Assessment of appropriateness of medication management for complications associated with the fall:
 - Bone integrity, including fracture and bone density (assess vitamin D and calcium status/consider supplementation).
 - Pain management.
 - Bleeding risk and anticoagulation management.
 - Fear of falling associated with medications.
 - Anticholinergic burden.¹²

The initial assessment and clinical review should include screening to determine the appropriateness and clinical need for a comprehensive falls medication review and be documented in the health care record of the patient.¹⁰

Appendix 5: Pharmacist Post Fall Guidelines (Pg2)

Comprehensive medication review

If a comprehensive medication review is needed, pharmacists may do this at the time of initial review or within two working days of the fall. Referral may alternatively be made for a timely medication review in the community, dependent on appropriateness and availability of services. The comprehensive medication review should include:

- Consideration of the patient's clinical condition, medication plan, and potential falls-related medication issues.⁹⁻¹¹ The approach outlined for 'the initial assessment and review' may guide this.
- Meaningful engagement with the patient or carers to guide recommendations if possible.¹³
- Development of appropriate evidence-based recommendations to the patient's medication plan.
- Communication of relevant changes to the medication plan to the MO, General Practitioner, patient, or carer, etc. documenting in health care record and educating of the patient or carer on medication self-management strategies.¹⁰

Appendix 6: Nursing/Multidisciplinary 48 Hour Post Fall Pathway

Nursing Guideline		DATE & TIME OF FALL:
NURSING/MULTIDISCIPLINARY 48 HOUR POST FALL PATHWAY		
Stop and Consider: Patients on anticoagulant, antiplatelet therapy and/or patients with a known coagulopathy (e.g. alcohol-dependent persons) are at an increased risk of intracranial, intrathoracic and/or intra-abdominal haemorrhage		
IMMEDIATE POST FALL PROCEDURE		
Complete DRSABCDE actions prior to moving the patient from the floor • Provide reassurance and comfort to patient and call for assistance. • Patient not to be moved if any physical injuries identified (unless airway is compromised) and fast track Medical Officer (MO) review within 30 minutes. (Regional sites consider Emergency Telehealth). • Activate Medical Emergency Team (or local process) if the patient meets criteria. • Immobilise cervical spine if the patient is unconscious or reports head or neck pain. • The patient's movement to be guided by clinical assessment and local policy/guideline. • Minimum investigations include: ▪ Blood glucose level ▪ ECG ▪ Cognitive impairment screening using the Abbreviated Mental Test (AMT4), Rapid Clinical Test for Delirium (4AT) or Confusion Assessment Method (CAM) (as per local policy/guideline). • Identify immediate pre-fall symptoms e.g., dizzy, feeling unsteady, etc. and consider other investigations as indicated by the pre-fall symptoms. • Notify MO of patient fall and request review. (If no apparent injury, this can occur within 4 hours or as per local policy). • Notify Ward/Area/Facility/ Senior Registered Nurse (SRN)/After Hours Clinical Nurse Specialist/Manager/MO. • Depending on type of fall, commence observations and care delivery as follows:		
WITNESSED FALL (NO HEAD STRIKE)	→	Commence neurological and baseline physical observations as per Medical/SRN's clinical judgment and document rationale.
PATIENT ON ANTICOAGULANTS / ANTIPLATELETS AND/OR WITNESSED FALL (HEAD STRIKE) or UNWITNESSED FALL	→	• Neurological observations: ○ Half-hourly for a minimum of 2 hours until GCS of 15 or patient considered back to their normal level of cognition achieved. ○ Continue if GCS remains < 15 or patient not considered at a normal level of cognition. Report to MO and continue as per instructions. • If the patient has GCS of 15 or patient considered back to their normal level of cognition, then continue observations: ○ Hourly for 4 hours. ○ Two-hourly 4 hours. ○ Four-hourly until a total of 48 hours from the time of fall has occurred). ○ At 48 hours if clinically assessed as stable, no clinical deterioration, document in medical record and return to observations pre-fall.
PATIENT ON PALLIATIVE PATHWAY	→	• Patients at the end-of-life that are no longer having observations: • The consultant or senior registrar must document in the patient's health care record whether observations are to restart or not.
GOALS OF CARE CHECK	→	• Review patients' goals of care to identify post fall care. The patient's treating Consultant or General Practitioner must guide this. • Patients on multipurpose sites may not need to be transferred to hospital for review.
RECOMMENDED ACTIONS WITHIN 4 HOURS:		
• Medical review of patient has occurred. • Identify the contributing factors to the fall and implement interventions to reduce the risk of another fall/injury.		

Appendix 6: Nursing/Multidisciplinary 48 Hour Post Fall Pathway (Pg2)

RECOMMENDED ACTIONS WITHIN 6 HOURS: Continue observations as indicated by fall type	
• Notify Next of Kin (NOK)/carer and document commencement of open disclosure processes (form or sticker) in the medical record. • Monitor for physical, behavioural, cognitive, and clinical deterioration. Report to MO if this occurs. • MO to have reviewed any urgent results of tests. • Refer/notify Occupational Therapist, Physiotherapist and Pharmacist of the fall. • Arrange referral to other health professionals as per clinical assessment. • Ensure contributing factors to the fall have been identified and interventions implemented. • Rescreen, (using local endorsed falls risk assessment tool), review current falls prevention interventions and implement additional interventions as required. • Document the fall in the medical record. • Complete official reporting via local system e.g., RiskMan, DATIX. • For an injurious fall that may be considered a SAC 1 injury – complete notification as per local clinical incident management policy.	
RECOMMENDED ACTIONS WITHIN 24 HOURS: Continue observations as indicated by fall type	
• Notify Medical Officer immediately if any changes in observations including visual changes, speech/language disturbance or focal motor/sensory changes. • Review of results of bloods, imaging, microbiology, and observations has occurred and been actioned. • Review fall, identify and implement interventions required for the patient's falls management care. • Patient and family/carers to receive information and education. Ongoing falls management care developed in partnership with patient and family/carers. • The multidisciplinary team members have collaboratively discussed the fall and identified any further risks and interventions required and these have been implemented. • Consider a structured multidisciplinary Post Fall Safety Discussion.	
RECOMMENDED ACTIONS WITHIN AND AT 48 HOURS:	
• At 48 hours post fall, review observations and if no clinical deterioration, return to appropriate observations. • Completion of all actions within the guidelines. • Conduct a comprehensive review of patient care. • Document in the inpatient record and communicate any outstanding actions and date/time completion required by multidisciplinary team members. • Consider a structured multidisciplinary Post Fall Safety Discussion within 48 hours of the fall.	
COMMUNICATION:	
• Ensure patient consents to discussion of care with family/carers (where clinically appropriate). • Interpreter is always to be utilised where appropriate (and as per local policy). • Primary nurse to ensure documentation in patient's health care record and local reporting database. • Medical and allied health reviews documented in the patient's health care record. • Patient and family/carers to receive information/education about the fall and ongoing instructions if discharged within 48 hours of the fall. (Provide "Health Information After a Fall" leaflet). • All disciplines involved are to partner with the patient and family and share decisions to develop ongoing plan of care. • Communication may require different approaches depending on disability/cultural requirements. • Documentation of the fall to occur on nursing, medical, allied health handover sheets, and all transfer and discharge documentation. Inclusion of the fall in verbal handovers: nursing, medical, allied health. • All staff involved in the care of the patient to be informed of incident outcome and revised care plan. • Visual flagging that the patient is at high risk of falls (and as per local policy). • Contact Ward/Area/Facility/SRN/After Hours Clinical Nurse Specialist (and as per local policy)	
ALLIED HEALTH ASSESSMENT: OCCUPATIONAL THERAPY, PHYSIOTHERAPY, PHARMACY	
• Complete assessments as per specific discipline guidelines within 2 working days of the fall (and as per local policy). • Work collaboratively with the wider multidisciplinary team.	

Appendix 7: MHS Functional Assessment Form (Older Adults)

 Government of Western Australia Department of Health Mental Health		FAMILY NAME _____ MRN _____					
		GIVEN NAMES _____ ☐ MALE ☐ FEMALE					
Site _____		D.O.B. ____/____/____ M.O. _____					
Mental Health FUNCTIONAL ASSESSMENT (OLDER ADULTS)		ADDRESS _____					
		LOCATION _____					
		COMPLETE ALL DETAILS OR AFFIX PATIENT LABEL HERE					
<i>This module assists the identification of functional abilities in an aged care consumer. It may be used at assessment, review and discharge. Please attach to relevant base module and summarise findings in relevant components e.g. if completed at assessment, document in 'Current functioning and supports'.</i>							
ACTIVITIES OF DAILY LIVING (please tick)							
Function	Independent	Initial set up/ preparation	1 person assist	2 person assist	> 2 person assist	Comment / use of aids	
Bathing							
Mobility							
Mobility in Bed							
Transfer							
Toileting							
Incontinence	Continent						
Eating							
Other (specify)							
Sleep	Pattern						
	Special requirements						
Nutrition	Special requirements						
Falls risk factors present (tick applicable factors)	☐ Previous falls	☐ Cognitive impairment	☐ Visual impairment	☐ Environmental factors	☐ Incontinence	☐ Medication effects	☐ Physical / medical
COMMENTS (consider strengths/skills & deficits etc)							

INSTRUMENTAL ACTIVITIES OF DAILY LIVING (please tick)							
Activity	Independent	Limited assist	Significant assist	Comment			
Money management							
Medication management							
Telephone use							
Travel / transport							
Shopping							
Cooking							
Washing							
Housework							
Maintenance							
Gardening							
Pet care							
Other (specify)							
COMMENTS (consider strengths/skills & deficits etc)							

Staff Name: _____		Signature: _____		Designation: _____		Date: _____	

Page 1 of 2

TMHMR910 MENTAL HEALTH FUNCTIONAL ASSESSMENT (OLDER ADULTS)

Appendix 7: MHS Functional Assessment Form (Older Adults) Pg2

[illegible]

TMHMR910 Page 2 of 2

Appendix 8: MHS Physiotherapy Post Fall Review Form (PMR18K)



NORTH METROPOLITAN AREA HEALTH SERVICE MENTAL HEALTH Hospital PHYSIOTHERAPY POST FALL REVIEW		Surname		Sex	U.R. No
		Forenames		D.O.B.	
		Address			
		Ward	Registrar	Consultant	
		Use Patient I.D. Label when available			
Identify	3pt ID confirmed: ☐ Verbally ☐ NS ☐ NOK ☐ Wristband ☐ Known to PT				
Situation	Date of fall: Informed by: Medical review post fall: ☐ Y ☐ N Date: Details of fall:				
Observations Vital signs, imaging	Reviewed imaging post fall: ☐ Y ☐ N/A Details if applicable:				
Background Pre admission mobility, Pre fall mobility, incl aid used	Pre-admission mobility documented in notes: ☐ Y ☐ N Date: Details: Pre fall mobility documented in notes: ☐ Y ☐ N Date: Details:				
Assessment Current mobility if appropriate, balance Ax if appropriate	Cleared to mobilise by medical team ☐ Y ☐ N If no, details: Bed mobility: STS: Ambulation:				

PMR18K PHYSIOTHERAPY POST FALL REVIEW

HCHGRFMR018K

G251
05/24

Appendix 8: MHS Physiotherapy Post Fall Review Form (PMR18K) Pg2)

NORTH METROPOLITAN AREA HEALTH SERVICE MENTAL HEALTH Hospital PHYSIOTHERAPY POST FALL REVIEW		Surname		Sex	U.R. No																	
		Forenames		D.O.B.																		
		Address																				
		Ward	Registrar	Consultant																		
		Use Patient I.D. Label when available																				
Assessment (Balance Ax if indicated, outcome measures if indicated)	Further physiotherapy assessments (e.g., balance, strength, functional tests):																					
	Any limitations to ongoing participation (e.g., pain, acute, confusion):																					
Recommendation Mobility / falls management recommendations (e.g., education, footwear modification, exercise programs, gait aids, staff specials, further referrals)	Falls Risk Factors identified:																					
	<table border="0"> <tr> <td>☐ Age > 65</td> <td>☐ Impaired vision</td> </tr> <tr> <td>☐ Acute medical condition</td> <td>☐ Wandering behaviour</td> </tr> <tr> <td>☐ Recent surgical intervention / post op</td> <td>☐ Environment / lighting</td> </tr> <tr> <td>☐ Previous fall</td> <td>☐ Footwear</td> </tr> <tr> <td>☐ PMHx</td> <td>☐ Malnutrition</td> </tr> <tr> <td>☐ >5 medications</td> <td>☐ Impaired cognition</td> </tr> <tr> <td>☐ Psychoactive medications</td> <td>☐ Dementia</td> </tr> <tr> <td>☐ Incontinence</td> <td>☐ Delirium</td> </tr> <tr> <td>☐ Reduced mobility / balance</td> <td>☐ Reduced insight</td> </tr> </table>					☐ Age > 65	☐ Impaired vision	☐ Acute medical condition	☐ Wandering behaviour	☐ Recent surgical intervention / post op	☐ Environment / lighting	☐ Previous fall	☐ Footwear	☐ PMHx	☐ Malnutrition	☐ >5 medications	☐ Impaired cognition	☐ Psychoactive medications	☐ Dementia	☐ Incontinence	☐ Delirium	☐ Reduced mobility / balance
☐ Age > 65	☐ Impaired vision																					
☐ Acute medical condition	☐ Wandering behaviour																					
☐ Recent surgical intervention / post op	☐ Environment / lighting																					
☐ Previous fall	☐ Footwear																					
☐ PMHx	☐ Malnutrition																					
☐ >5 medications	☐ Impaired cognition																					
☐ Psychoactive medications	☐ Dementia																					
☐ Incontinence	☐ Delirium																					
☐ Reduced mobility / balance	☐ Reduced insight																					
Details:																						
Mobility recommendations:																						
Falls risk management recommendations:																						
Falls education provided to patient and support staff:																						
Follow up physiotherapy required: ☐ Y ☐ N																						
Name:		Signature:		Date:																		
Designation:																						

Appendix 9: Flowchart for Home Visiting Health Professionals

Patient Falls at Home

(includes witnessed or unwitnessed falls at home or in community in last 72 hours)

Assess

- Reassure patient
- Perform D.R.S.A.B.C.D
- Administer First Aid within skill limit
- Check for injury(*)
- Move patient if required for safety(*)
- Make comfortable until help arrives

Escalate Care?

- Loss of consciousness/ Evidence of seizure
- Noted change in cognition/behaviour(*)
- Hit head or visible evidence Head Injury.
- Signs of Injury
- Pain or symptoms of clinical concern
- Use of Anticoagulation medication(*)
- Unable to move from floor safely or without assistance(*)

Seek Assistance

- General Practitioner / Locum service
- Paramedic/ Ambulance(*) CALL 000
- Emergency Department
- Hospital/ ED Avoidance Programs
- Local Health Service Medical Officer
- Anticoagulation Nurse Practitioner(*)

***See over for additional information**



Government of **Western Australia**
Department of Health

Multidisciplinary Post Fall Flowchart for Home Visiting Health Professionals

Good Practice Points

Remember

- Provide patient/ carer "**Health advice following a Fall-Community**" brochure. Review information as appropriate.
Visit: <https://www2.health.wa.gov.au/Articles/F/Falls-prevention-and-management-in-WA>
- Document fall in patients medical file(PTO for documentation prompts and criteria for completion of "CIMS form")*
- Consider referral for management of their ongoing falls risk.
Visit: <https://www.stayonyourfeet.com.au/health-professionals/referrals-to-health-professionals/> or access it directly here **SOYFWA**

Communication

- Advise your immediate supervisor/ manager of incident
- Obtain patient consent as able prior to initiating care
- Contact Next of Kin, Carer/ Care provider
Alert them to fall. Request visit I review if possible
- Contact patients GP. *Alert them to fall*
- Liaise with Multidisciplinary team
- Contact patient later in the shift or next day to check well-being.

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Appendix 10: Accompanying Information for Home Visiting Health Professionals



Government of Western Australia
Department of Health

Multidisciplinary Post Falls Flowchart for Home Visiting Health Professionals:
Accompanying Information

This flowchart should be used by home visiting health professionals (HP) to guide their assessment and management of patients who have fallen at home within the preceding 72 hours *and not received medical assessment*. It is not intended to take away from clinical judgment. HP should work within their Scope of Practice when managing the patient who has fallen. If there are any concerns with acute management of a fall or ongoing management of their falls risk, they should be referred to an appropriate health service as outlined in the flow chart. For long term management of falls risk refer to the [Stay on Your Feet Health Professional](#) portal for a list of services available in WA. Refer to [WA Post Falls Guidelines 2023](#) for further information.

DATIX CIMS notification. A CIMS form should be completed when a fall occurs whilst the HP is present or the patient is on floor when the HP arrives at the patient's house. Notification of a clinical incident is made online using the Datix CIMS application or if electronic access is not available via a hard copy clinical incident form (contact your line manager). Further details re CIMS reporting can be found in the [WA DOH CIMS Policy Pack \(CIMS Policy 2019; CIMS Guidelines 2019; CIMS Toolkit 2019\)](#).

Documentation Prompts. Circumstances of fall (what, how, when), associated symptoms occurring at time of fall (e.g. dizziness, light-headedness, syncope, vertigo, weakness, incontinence, confusion / disorientation), associated falls related injuries, was the fall witnessed, environmental hazards, medical / nursing / allied health / First Aid treatment required and delivered. Include any communication performed to report the fall.

Checking for Injury. Encourage the client to stay calm and avoid moving or trying to stand whilst you undertake your assessment. Is the patient complaining of pain in the neck or head region? Can the patient move their upper and lower limbs without pain? Is there any new bruising, swelling or deformity? Has the patient's mobility changed as compared prior to the fall? Has the patient's conscious state (e.g. alertness, arousal, appropriately following commands) or cognition (e.g. memory, reasoning and orientation) changed? *Note:* Any alteration in a patient's conscious state or cognition may impact on their capacity to make decisions regarding their ongoing care including the need for further medical intervention after a fall. HP should consider further assessment if there are concerns about an acute change in the patient's cognition (e.g. Delirium) using a recognised tool such as the [4AT](#) (inclusive of AMT4) or communicate with the client's GP or primary carer for further clarification regarding the patient's baseline cognitive level if there is any uncertainty.

Manual Handling - moving patient's safely. All staff are requested to follow local OSH guidelines during lifting and transfer of patient loads. Staff should consider their individual training and clinical skills prior to deciding whether to assist a patient on the floor. Staff should only consider moving a patient from the floor if:

- Risk assessment indicates that they are able to assist without injury to the patient or themselves at all times.
- Safe manual handling techniques and postures are maintained. Please refer to the local OSH Manual Handling guidelines for a description of an appropriate technique in a domiciliary setting. Staff can download the [Stay on Your Feet Up from Floor Poster \(Arms or Knees\)](#) for people able to perform the floor transfer independently, via prompting or with supervision.
- The patient needs to be removed from immediate danger.

When to call the ambulance. If a person falls at home or in the community and cannot get up independently or with minimal assistance and / or if urgent medical assistance is required an ambulance should be called (please refer to local policy and guidelines). In addition, consideration should be given to other assessment findings such as pain of clinical concern (including neck or head pain), altered consciousness, confusion, significant injury requiring medical attention or clinical judgement suggests that there is concern over the patient's wellbeing.

Communication. Staff should attempt to make contact with the patient's GP and primary carer to alert them to the fall, consequences of the falls and any interventions delivered or required. Patient consent should be obtained where possible. Staff should ensure that the patient has appropriate capacity to make an informed decision (*Refer to Checking for Injury above*) in the event that they refuse medical attention or refuse to have their GP / primary carer alerted to the fall. If the patient has capacity to make a decision, staff should abide by the patient's request. However if the staff member is uncertain about the patient's capacity either due to pre-existing medical conditions or as a result of the fall (e.g. hit head) then contacting the patient's GP to assist in determining capacity and the need for medical assistance may be justified if it is in the patient's best interest. Staff should seek further assistance by contacting their line manager.

The fallen patient who has Coagulopathy or is on prescribed Antiplatelet and or Anticoagulant medication. Patients who are coagulopathic, on antiplatelet and or anticoagulant medications are at an increased risk of bleeding related injuries after a fall. Common medications include: Warfarin, Clexane, Rivaroxaban, Dabigatran, Apixaban, Aspirin Clopidogrel, Ticagrelor, Asasantin SR, Dalteparin ([A detailed list has been produced by QLD Health](#)). If the person does not require transport to Hospital, an urgent visit to their GP is recommended due to the risk of undiagnosed internal bleeding. If patient is under the management of an Anticoagulation Service contact the service provider for further advice regarding management.

DISCLAIMER: This document has been developed with care but may not be appropriate for all clinical scenarios. Clinical assessment along with medical assistance and advice must be sought if in any doubt.

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Further information can be obtained from
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